

العمر(تاريخ الميلاد) : 27-06-1982 المهنة : ربه منزل

ملاحظات : ---

المجلس الطبي المتخصص

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Name: ولاء عبدالمعبود علي محمد National ID: 28206271400988	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

B 200

جمهورية مصر العربية
بكالوريوس الحقوق الشخصية

علام

عبدالمعز علي محمد علم الدين

ميت فسا

مركز قنوب - القليوبية

١٩٨٢/٠٦/٢٧

٢٨٢٠٦٢٧١٤٠٠٩٨٨

KG3491243

Name: ولاء عبد المعبود على		
MR. : 1030		
Detailed Medical Report		
		السيد
تحية طيبة وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالتها		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انها The patient’s tumor was documented as estrogen receptor (ER) 8/8, progesterone receptor (PR) 8/8, and HER2 negative. Following neoadjuvant chemotherapy (POST NACTH), the patient underwent radiation therapy after breast-conserving surgery (RT BCS) with pathology indicating PTIS PNIS M0. Multidisciplinary team discussion in February 2022 recommended a regimen of PORTH, ZOLADEX, and TAMOXIFEN; the patient is currently receiving ZOLADEX and TAMOXIFEN. A pelvic-abdominal ultrasound performed in April 2026 was reported as free of abnormal findings. A sonomammography for tamoxifen monitoring (3 months) and supportive care (3 months) remains pending. The patient is scheduled for the next follow-up visit in five weeks.		
تحريراً في ,		
	التاريخ :	18-09-2026
	الوقت :	نهى سعيد علي محمد
الطبيب المعالج :		
يعتمد ,		

Patient Name : الأستاذ / ولاء عبدالمعبود على محمد علم الدين	Request No. : 13004076
Patient No. : 28206271400988	Request Date : 05/12/2022
Path No. :	Verification Date : 18/12/2022
Sex : Female	Financial Category : اورام دار السلام - مبانزه
Age : 40 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Right breast cancer, post NACT.

Nature of the specimen: Right BCS and Right SLNs biopsy.

Gross Examination:

- Procedure: Right breast coservative surgery specimen; measured 10.5x7x3 cm, Covering nipple and areola measured 4x2 cm.
- Tumour extent : An ll defined rubbery grayish white tumor bed site
- Size: 5x3x2 cm
- Distance from margins: 3 cm from lateral and medial margins, 0.4 cm from deep margin (least).
- Lymph nodes number and size: Four nodules up to 1x1 cm
- Frozen section: Free margins, One lymph node suspicious for micrometastasis.

Microscopic examination.

- Histological type: Ductal carcinoma insitu, no invasive component
- Angioinvasion: Negative
- Lymph node involvement: One positive for micrometastasis
- Resection Margins: Free
- Histologic response to therapy: Pathologic partial response
- Additional pathologic findings: Free nipple and areola

Diagnosis:

Right breast cancer; Right BCS and SLNs biopsy:

- Ductal carcinoma insitu, with no invasive component
- Free nipple and areola
- Free surgical margins
- One lymph node shows micrometastasis

Verified by :

dr.esra a magdy

Printing Date : 20/12/2022 09:24:38

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Printing By : marwa mohamed youss

Patient Name : الأستاذة / ولاء عبدالمعبود على محمد علم الدين	Request No. : 13004076
Patient No. : 28206271400988	Request Date : 05/12/2022
Path No. :	Verification Date : 18/12/2022
Sex : Female	Financial Category : مبانره - اورام دار السلام
Age : 40 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

- Partial pathologic response
- ypTis ypN1mi ypMx

Prof.Dr.Manal Elmahdy
Dr.Israa Elgendy

Verified by :
dr.esraa magdy

Printing Date : 20/12/2022 09:24:38

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Printing By : .marwa mohamed youss



Patient Name	: الأستاذة / ولاء عبدالمعبود علي محمد علم الدين	Request No.	: 13008217
Patient No.	: 28206271400988	Request Date	: 18/04/2022
Path No.	: 1040/2022	Verification Date	: 10/05/2022
Sex	: Female	Financial Category	: اورام نادر السلام - مياطرة
Age	: 40 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: dr.Enas

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:

Right breast UOQ lesion.

Nature of the specimen:

Tru cut biopsy, collected 17/4/2022, 11:10 AM.

Gross Examination:

- Multiple grayish white tissue cores up to 1 cm, totally submitted.
- Sections prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9)

Microscopic examination:

- Histological type: Invasive duct carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis): II(3+2+2)
- With DCIS component
- Biomarkers:

ER: Strong nuclear reaction in 95% of tumor cells (Allred score 8/8)

PR: Strong nuclear reaction in 95% of tumor cells (Allred score 8/8)

Her-2: No membrane reaction in tumor cells (score 0)

Ki 67: 15 % nuclear reaction

Diagnosis:

Right breast lesion; Tru cut biopsy:

- Invasive duct carcinoma, grade II
- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- HER-2: Negative (score 0).

Verified by :

walaa ghoniem

Printing Date : 14/05/2022 09:43:49

Page 1 of 2

Printed By : Ali mostafa



Patient Name: Walaa Abdel Ma'boud Ali Mohamed	Date : 23/04/2024
Exam : C.T CHEST WITH CONTRAST	Age : 41 Year(s)
Medical ID: 1793/2022	Pat. ID: 13113

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

- Reported case of operated breast cancer.
- Compared to the previous study dated 30-3-2023.

Chest:

- Right breast & axillary operative beds showing no sizable masses or sizable axillary lymph nodes.
- Still noted right upper lung lobe posterior segment pulmonary nodule with internal calcification, mostly representing post granulomatous sequel.
- Currently noted right upper lobe and middle lobe fine subpleural reticulations, likely post therapeutic sequel.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

Abdomen and pelvis:

- The liver is still moderately enlarged with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- IUD is seen.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

The scanned skeleton:

- Stable L2 small and right iliac tiny lytic osseous lesions.



Total DLP (mGy.cm):

Patient Name: Walaa Abdel Ma'boud Ali Mohamed	Date : 23/04/2024
Exam : C.T CHEST WITH CONTRAST	Age : 41 Year(s)
Medical ID: 1793/2022	Pat. ID: 13113

- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- Right breast & axillary operative beds showing no sizable masses or sizable axillary lymph nodes.
- Rather stationary rest of the study.
- No definite pulmonary or hepatic metastatic deposits.

Dr. Ali Elsisy, MSc.

Signature Dr. Mohamed Mohsen

29/04/2024 08:46



Patient Name: Walaa Abdel Ma'boud Ali Mohamed	Date : 12/09/2024
Exam : MAMMOGRAPHY BOTH	Age : 42 Year(s)
Medical ID: 1793/2022	Pat. ID: 13113

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Patient had rt CBS

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- No left breast suspicious mass lesions, micro calcifications or architectural distortion detected.
- Right breast focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound showed right breast UOQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- no left breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right infra and supra clavicular regions.

Opinion:

- Right breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- Normal study of the left breast (BIRADS 1)

Yours truly

Signature Dr. Asmaa Zenhom

12/09/2024 3:10 PM



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الاستراتيجية
صحة
مصر

MDT FORM 24/11/2022 G

Name	محمد عبد الحليم	MRN	1493155
Age	40 y	ID	CAC-76715599A
Menopausal status		Phone	0158.176370
NRS SCORE	1	Marital status	0187.47997
BMI	over weight	Family history	
CO morbidity		REASON OF MDT	

Clinical Data

BX:	IDC			R	L
ER:	2/4	PR:	2/4	HER 2:	-ve
Ki67:	15%				
Sonomammography:	CT3 No				
Mets Workup:				yctNM0	

Recommendations:

elad LAC - cut as bre bot 7/11/2022
Planned Rt skin spx planned SLND
for MDT → free

MDT OUTCOME & PLAN

For upper Quadrantectomy +
Reconstruction by mini LD
+ SLND

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric